Weekly cetuximab + head and neck radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Weekly cetuximab with radiotherapy

Active medicines: Cetuximab

Emetogenic risk: minimal

Medicine	Dose / route	When
Antihistamine and corticosteroid premedication	Clinician/pharmacy to complete	Before the first cetuximab infusion; strongly recommended for subsequent infusions Complete the local authorised prescription and observation plan.

Additional phase-specific support

- Ask for one schedule showing prevention medicines, rescue treatment and what to do if symptoms persist. Do not add, double or restart medicines from an old prescription.
- Premedication helps reduce infusion reactions; the team also provides a skin-care plan. Keep mineral blood-test appointments, including monthly checks for two months after the final dose. Report delayed reactions even after earlier infusions were tolerated.
- A dietitian can tailor food texture, supplements and calorie or protein needs. Tell the team early if eating or drinking becomes difficult; do not wait for substantial weight loss. Fluid targets depend on kidney and heart health.
- Use the mouth-care plan provided by the team and tell them about ulcers, thrush, bleeding or pain. Inform your dentist about cancer treatment. Before any dental procedure, confirm timing with the oncology team.
- A speech and language therapist can assess swallowing and advise exercises or safer food and drink textures. Follow the individual assessment; a general guide cannot decide which textures are safe for you.
- Discuss fertility and pregnancy precautions before treatment and the medicine-specific duration of contraception afterwards. Ask about breastfeeding. Bring a full medicine, supplement and allergy list; check before adding over-the-counter or herbal products.
- NCCP v6 permits ending post-infusion observation after two tolerated doses; EMA Erbitux specifies at least one hour after every infusion. The clinician/pharmacy team must reconcile this difference locally; no automated early-discharge instruction.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00207 v6 · pSee regimen treatment, tests and supportive-care sections · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-07 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required